

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME:** **5 MINUTES**
WRITING TIME: **40 MINUTES**

Read the case notes below and complete the writing task which follows.

Notes:

Mr Bruce Smith is a regular customer of yours. He has a history of gout and hypertension. One week ago, he commenced oral antibiotics to treat a respiratory infection. Mr Smith saw his doctor this morning for a check-up and has come to you with a prescription.

Patient's name: Mr Bruce Smith

DOB: 14/02/1963

Residence: 28 Park Avenue, Lakeside

Social background: 52-year-old accountant

Medical history: Gout (2 years)
 Hypertension (6 months)
 Current respiratory infection

Current medications:

Probenecid 500mg tablets (gout treatment), 1 b.d.
Metoprolol (Betaloc) 50mg tablets (beta-blocker), 1 b.d.
Amoxicillin 500mg, 1 t.d.s.

Date of visit: 20 September 2015

Reason for visit: Client saw Dr for check-up this morning
 – BP elevated (155/95)
 – Dr prescribed increased dosage of BP medication
 – repeat course of antibiotics also prescribed

Medication required today:

Metoprolol (Betaloc) 100mg tablets (increased from 50mg), 1 b.d.
Amoxicillin 500mg, 1 t.d.s.

Discussions in pharmacy:

Client complains of unusual fatigue and insomnia (4–6 months).
Stomach aches during last week – client didn't mention these to Dr.
Client has busy lifestyle and stressful job – frequently forgets to take metoprolol tablet in evening, 'compensates' by taking two tablets the next morning.

Drug therapy Issues:

1. Non-compliance to metoprolol regimen
2. Sleep disturbance and lethargy – ?caused by metoprolol
3. Stomach ache – ?caused by probenecid, ↑levels of amoxicillin in blood

Immediate therapy goals:

Eliminate stomach pain
Maintain BP control

Management plan: Contact Dr and refer client with the following recommendations:

- Replace metoprolol Betaloc with metoprolol Toprol-XL long-acting once-daily dose
- Suggest replacing amoxicillin with cefaclor (second-generation cephalosporin), 250mg every 8 hours

Writing Task:

Using the information given in the case notes, write a letter to Mr Smith's medical practitioner, Dr Richard Black, 33 Gardiner Rd, Lakeside. Outline your concerns with Mr Smith's medication regime and provide appropriate recommendations.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

You are a pharmacist at Newtown Pharmacy.

PATIENT DETAILS:

Name: Mrs Adela Barta
DOB: 25.07.1990
Address: 515 Railway Street, Newtown
Social history: Married: Husband – Mr Alexander Barta
Work: School Teacher

Medication history:

14/07/13 First visit to this pharmacy
TheraTears Lubricant Eye Drops (carboxymethylcellulose sodium) – artificial tear solution for dry eyes
19/07/13 Bleph-10 eye drops 100mg/mL (sulfacetamide sodium) – antibiotic for conjunctivitis
21/07/13 Zylet 20mL (loteprednol/tobramycin) – steroid/antibiotic eye drops for conjunctivitis
26/10/14 Finger Splint – sprain index finger R hand
22/08/15 Bactrim Tablets (sulfamethoxazole/trimethoprim)

Referrals:

General Practitioner (GP): Dr Claus Becker
Optometrist: Ms Tilly Gallagher
Physiotherapist: Ms Sandra Rios
Allergies: No known allergies

OET Victory with Dr Ashgan

Treatment Record

14/07/13	Client presented with symptoms of dry eyes, possibly due to overuse of computer. Recommended TheraTears and suggested regular rests from looking at computer screen. If no improvement see optometrist.
19/07/13	Client presented with conjunctivitis. Optometrist prescribed Bleph-10 eye drops.
21/07/13	Client returned. No improvement in conjunctivitis. Increased itching and burning. ?Allergy to sulfonamide antibiotics. Zylet topical steroid/antibiotic drops prescribed by optometrist.
26/10/14	Finger Splint (lightweight aluminium, padded on one side) recommended by physiotherapist. Client reports Zylet worked well/no side effects.
22/08/15	Bactrim prescribed by GP for urinary tract Infection (according to client). Concern about previous reaction to Bleph-10. ?allergy to sulfonamide antibiotics. Advised client of potential reaction to Bactrim – client not happy about waiting.
Plan:	Letter to GP to suggest different antibiotic – GP not currently available on the telephone. Stress in letter that client wants response ASAP. Will advise Mrs Barta when new med is available to collect.

Writing Task:

Using the information provided in the case notes, write a letter to Mrs Barta's GP, Dr Becker, suggesting a review of the prescription and detailing the client's relevant medication history. Address the letter to Dr Claus Becker, Newtown Medical Clinic, 72 Main Street, Newtown.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

You are a pharmacist at a local pharmacy. Ms Katrina Morrison presents you with a prescription for her husband who is sick at home with a lower respiratory tract infection.

Date: 21 January 2015

Dr D Baldwin
1 Hope St
Riverview

Mr Erwin Morrison
200 Garden Rd
Newtown

Amoxycillin 500mg tds. 20 capsules 1 rpt.

Signed: *D Baldwin*

Patient: Mr Erwin Morrison (D.O.B.: 01/09/1956)
Residence: 200 Garden Rd, Newtown
Social background: Semi-retired academic
Current medications: Lipitor (atorvastatin) 20mg mane
Drixine nasal spray (oxymetazoline hydrochloride 0.05%)
Indocid (indomethacin) 25mg prn
Colgout 0.5 (colchicine) 500mcg prn
Medical history: Hypercholesterolaemia (8 years)
?Gout (10 years)
Recent History: 3 days ago Pt went straight home to bed without filling out antibiotic script (21 Jan).
Dr instructions: take course of antibiotics then ring to discuss possible repeat.

Drug therapy problems:

Nasal spray overuse:

- Pt likes using nasal spray to relieve congestion – used for 1yr on and off, often for many weeks in a row. Pt's wife knows – advises against.

Medication compliance:

- Pt very poor at taking medications (esp. antibiotics), takes medication as he pleases/does not follow instructions.
- Lipitor compliance: taken irregularly until Dr highlighted the risks (e.g., high cholesterol).

Discussions with Pt's wife:

Amoxycillin

- Highlighted importance of commencing course & completing full course.
- Can take on empty or full stomach.
- One capsule should be taken regularly at 8hr intervals.
- Confirmed wife – Pt no penicillin allergies.
- Contact Dr if symptoms get worse, experience diarrhoea or tongue becomes white coated/sore.
- Must not use expired antibiotics.
- Consult Dr when finished first course.

Drixine nasal spray (oxymetazoline hydrochloride)

- Nasal spray should only be used short term (~ 3-5 days).
- Risk damaging nasal epithelium (nasal lining), exacerbate rhinitis medicamentosa (e.g., recurring rhinitis, immunity to treatment).
- Discussed effects of long-term use – permanent turbinate hyperplasia (proliferation of nasal cells).
- Suggestions: Pharmaceutical saline nasal spray to wean off Drixine nasal spray. If no result cortisone may be prescribed by Dr.

Recommendations/Management plan:

- Pt's wife concerned – wants support to convince husband to follow Dr's orders (begin & complete antibiotics course).
- Discussed amoxicillin consumer medication information brochure.
- Provided information on safe use of nasal sprays.
- Offered to write letter to Pt.
- Recommended discuss with Dr.

Writing Task:

Using the information in the case notes, write a letter to Mr Erwin Morrison, 200 Garden Rd, Newtown. Outline your concerns with his prolonged use of nasal spray and the importance of following his antibiotics regime strictly.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME:** **5 MINUTES**
WRITING TIME: **40 MINUTES**

Read the case notes below and complete the writing task which follows.

Notes:

You are a pharmacist at a local pharmacy where Mrs Aoki is a regular client. She has received prescriptions from several doctors.

CLIENT DETAILS:

Name: Mrs Tomomi Aoki

DOB: 22.08.1994

Address: 923 Sand Street, Newtown

Social history: Married – husband Mr Dai Aoki

Medication history:

2010	Novasone ointment 0.1% (mometasone furoate) for atopic dermatitis that worsens with hot weather.
2011	Avamys nasal spray (fluticasone furoate) for allergic rhinitis – seasonal hay fever that worsens with hot weather. Novasone ointment 0.1%.
2012	Avamys nasal spray. Novasone ointment 0.1%.
2013	Avamys nasal spray. Novasone ointment 0.1%.
06/2014	Clomid (clomiphene) – selective estrogen receptor modulator (SERM), for ovulation induction.
09/2014	Prenatal vitamins (folic acid, iron and calcium). Vitamin B6 (pyridoxine) 10mg.
10/2014	Repeat vitamin B6 (pyridoxine) 10mg.

Referrals:

GP (since 10/2014) Dr Lucy Wu
Newtown Medical Clinic, 77 Main Street, Newtown

GP (from 03/2010-10/2014)
Dr Ian Williams
Newtown Private Practice, 90 High Street, Newtown

Fertility Specialist Dr Marco Villanova
Newtown Fertility Clinic, 89 Green Street, Newtown

Obstetrician Dr Phillipa Gou
100 Seaview Road, Newtown Hospital, Newtown

Allergies: Penicillin, surgical stainless steel and some metals.

Treatment Record

17/06/14	Client prescribed Clomid (clomiphene) 50mg (SERM) by Dr Villanova. Diagnosed with infertility, wants to conceive. Directions: Take 1 tab daily from days 3-7 of menstrual cycle. Client advised of side effects: hot flushes, visual disturbances, ovarian cyst and multiple pregnancies.
13/09/14	Client advised of pregnancy and now wishes to purchase best vitamins. I gave advice re: best vitamins to take in pregnancy. Client stopped taking Novasone ointment 0.1% due to pregnancy. Client mentioned she is unhappy with current GP (Dr Williams), wanting to change.
18/09/14	Client experiencing ↑ morning sickness – asked me about natural meds. I advised: • Vitamin B6 (pyridoxine) 10mg – client will consult GP before commencing. • Eat healthy diet (may need to eat small amounts at a time). • Take prenatal vitamins before bed.
19/09/14	Client returns with prescription from GP (Dr Williams) for vitamin B6 (pyridoxine) 10mg. Directions: 25mg 3x daily for 72hrs, then 10mg 3x daily for 7 days, then 10mg daily.
19/10/14	Repeat prescription for vitamin B6 (pyridoxine). Client reports B6 working well.
29/11/14	Contacted by client's new GP (Dr Lucy Wu). Client has flu. GP wants history of medications & vitamins prescribed to date.

Writing Task:

Using the information provided in the case notes, write a letter to Mrs Aoki's new GP, Dr Wu, informing her of the client's medication history. Address the letter to Dr Lucy Wu, Newtown Medical Clinic, 77 Main Street, Newtown.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

A diabetic client, Mr Davidson, is planning to travel abroad alone and has sought advice at your suburban pharmacy. He is quite anxious and is even wondering whether it is safe for a diabetic to travel.

He is concerned about several issues: problems with customs and security at the airport, not being allowed to take his medication on board the plane, not being able to carry on food/drink or get adequate food on board during a long flight, what to do if he loses his medication, and whether changes in climate will affect his medication and equipment.

Because of his rather anxious state, you decide to write down your advice and reassurances in a letter to the client.

Notes from a seminar you attended on travelling with medication

- | | |
|-----------------------------|--|
| Before you go: | <ul style="list-style-type: none">• Get letter from your doctor (several copies) explaining medication, devices/equipment• Get spare prescription• Arrange travel insurance (worthwhile, despite extra cost)• Plan with healthcare worker when to take insulin/eat during flight• Tell airline about your diabetes when booking• Order meals with adequate carbohydrate content |
| What to take: | <ul style="list-style-type: none">• Adequate supplies for the entire trip (and extra in case of loss/damage)<ul style="list-style-type: none">- Take medications in packaging dispensed by pharmacy- Labels should be clear on medications• Adequate pump batteries for insulin pump (where relevant)• Small first aid kit• National healthcare card• Quick acting carbohydrate (e.g., glucose tablets, jelly beans)• Small sharps container for syringe disposal (where relevant) |
| At airport security: | <ul style="list-style-type: none">• Carry all diabetes supplies for flight in hand luggage• Answer questions about your equipment• No need to declare diabetes equipment• Show letter from doctor• Insulin should not be affected by X-rays (Ask for hand-checking of carry-on luggage if concerned) |

OET Victory with Dr Ashgan

To keep blood glucose stable on the flight:

- Wear comfortable, loose shoes
- Eat regularly
- Ask for lemonade if feeling hypo
- Do some exercise
- Take regular doses of insulin (where relevant)
- No alcohol
- Plenty of water
- Sleep whenever possible
- Do not take insulin until meal has been served

Where to go if you lose your medication when travelling:

- Hospital emergency department
- Local diabetes organisation

Precautions:

- Wear identification that states you are diabetic
- Carry written details of your next of kin, medications and devices you use
- Store insulin and blood glucose test strips below 30°C, and above 0°C
- Check blood glucose meter for operational temperature range
- Check blood glucose levels more often

Writing Task:

Using the information given above, write a letter to Mr Davidson, 111 Cedar Street, Stillwater, addressing his specific concerns.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES

WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:

You are a pharmacist at Newtown Hospital. An elderly patient who has been treated for a fractured femur is being discharged. You are writing a letter to her carer (her daughter) to ensure the medication regime is followed when she returns home.

Patient History

Name: Mrs Alice Ramsey

Date of Birth: 4 January 1925

Allergies: Nil

Current Medication:

On Admission: Zantac (ranitidine) (for GORD): 150mg bd

Lipitor (atorvastatin): 20mg mane (on empty stomach)

On Discharge: Zantac (ranitidine) (for GORD): 150mg bd

Lipitor (atorvastatin): 20mg mane

Heparin low molecular weight (LMWH) (anti-coagulant): 7500 bd – to be continued until mobile

Panadoline Forte (paracetamol & codeine for pain relief): 500mg 4-hourly/prn

Durolax (to prevent constipation): 10mg nocte

Maxolon (metoclopramide) (for side effects of codeine): 10mg tds/prn

Penicillin (prophylactic: ↓ risk of post-op infection): 250mg qld 2/52

Drug Information: Adverse Drug Reactions

Ranitidine Adverse: headache; GI upset; rash; CNS disturbances (rare)

Atorvastatin Adverse: Serious: rhabdomyolysis, myopathy, myalgia (0.2%);
GI upset (1%); headache (2%); rash (2.5%);
flu-like symptoms (1.5%); raised LFTs (1.3%)

Heparin Adverse: haemorrhage, easy bruising, nausea, vomiting

Codeine/Paracetamol Adverse: constipation, stomach-aches, nausea, vomiting;
Rare: dependence, tolerance; CNS disturbances incl. impaired alertness

Metoclopramide Adverse: CNS disturbances incl. impaired alertness (rare); tardive dyskinesia (rare)

TURN OVER

Penicillin *Adverse:* Rare: sensitivity reactions; haematological effects; nausea, vomiting, mild diarrhoea; allergic skin rash or hives

Social History:

Pt normally lives alone. On discharge, staying with daughter.
Pt non-driver. Public transport.

Relevant History for Surgical Procedure:

Height 168cm; Weight 75kg; BMI 26.8

Non-smoker

Dentures – upper & lower

Gastro-oesophageal reflux disease (GORD) – controlled by medication

Hypercholesterolaemia – controlled by medication

11 July 2010

2:45pm: Pt brought to A&E by ambulance. Knocked down by car in Garden Nursery car park (buying plants) – landed on bitumen. Driver failed to see her in rear-view mirror → reversed into her. Not run over. Fell on R side on femur.

Presenting symptoms: pain – post fall & difficulty standing or walking

3.00pm: Pt seen by Dr Hogarth. Pain relief: pethidine (opiate)

X-rays of affected femur – anterior-posterior & lateral views

Repeat films with hip at 15-20° internal rotation → MRI

5:30pm: Transferred to ward

Pt booked for surgery 12 July am – nil by mouth from midnight

Full pre-operative general investigation: LFTs, platelet count, WBC count, WBC types, RBC count, RBC indices, Hg, haematocrit, blood smear, ECG & chest X-ray

12 July 2010

Open reduction & internal fixation (ORIF) performed

GA given: induction – propofol; sevoflurane, fentanyl, midazolam, suxamethonium, ondansetron

Heparin – thrombus prevention

IV antibiotics – penicillin – continued for 24/24 post surgery

Immobilised with splica cast

Post-Op

TURN OVER

OET Victory with Dr Ashgan

- pressure sore prevention & care of pressure areas; wound care
- pain relief
- fluid balance & blood loss monitoring: IV fluids + penicillin
- nutritional management: oral protein supplementation
- thrombus prevention: low dose, low molecular weight heparin, & mechanised compression stockings
- lower limb circulation & sensation
- early mobilisation & weight bearing on injured leg

24 July 2010 Transferred to Rehab Unit

8 August 2010

Due for discharge home – appointment made for 22 August 2010 for removal of cast
Letter to carer/daughter (NB: heparin to be continued only until mobile)
The patient is being discharged to the care of her daughter.

Writing task:

Using the information in the case notes, write a letter to the daughter, Mrs Holly Kerr, 3 Rose Avenue, Springbank, outlining her mother's medication regime, any potential adverse effects to be aware of, and when to seek medical advice.

In your answer:

- expand the relevant notes into complete sentences
- do not use note form
- use letter format

The body of the letter should be approximately 180-200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

Personal Details:

Name: Alexia Rollinson (Ms)
Address: 15 Fine St, Newtown
DOB: 12/11/1973
Age: 40 years
Date: 10 February 2014

Social/Family Background:

Single. Works full time as an accountant

Diagnosis: Hypertension, hypercholesterolaemia, low vitamin D since 2011

Medication: Betaloc (metoprolol), 100mg b.d.
 Lipitor (atorvastatin), 20mg mane
 Ostevit-D 1000IU mane

Current Status: BP 147/100mmHg (taken in pharmacy)
 Lipid profile: LDL – 131, HDL – 64, Triglycerides – 269mg/dl
 Vitamin D < 54 (60-160nmol/L) (print out with customer)
 Ht 153cm, Wt 65kg, BMI 27.8 (verbal from customer)
 Does not regular exercise – drives to work, no sport or recreational activity
 Low mood
 Overweight

Discussions in Pharmacy:

New to area, moved 1 month ago, and has no GP yet.
 Medications required today and repeats are filled.
 Came in for advice and explained current needs.
 Monitoring diet to decrease Wt – target 58kg, BMI <25.

- Exercise – Started own exercise program (e.g., walk 30 min 4 times/wk). Says 'never sticks to it'. Has tried all types of exercise aids advertised on TV, video programs, getting desperate & upset. Wants some help due to lack of progress.

- Diet – Discussed fruit & vegetables, low fat milk, low GI foods & low saturated fats. Bought two electronic scales last week, one for kitchen (food) & one for bathroom (self). Discussed fruit & nut snacks, not chocolate bars (admitted to loving them). Always browsing for Wt loss products. Tried several tablets, drinks, powders, etc. Getting desperate & upset. Wants help due to no progress with Wt loss or change in exercise & daily activities.
Offered to write to local GP for support. Also mentioned a dietitian – customer liked idea.

Pharmacy Management:

- Provided free booklets
 - Healthy eating and exercise
 - Council brochure on walking tracks, walking groups, etc.
 - Local gymnasiums & sports groups
- Letter to GP – suggested referral to dietitian

Writing Task:

Using the information in the case notes, write a letter of referral to Dr Sally Windwood, 9 Blewston St, Newtown, to explain your discussion and advice including a suggestion of consulting a dietitian.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

Occupational English Test

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:

You are a pharmacist in a rural community pharmacy. A teacher at the local primary school approaches you about an outbreak of head lice at the school. She asks you to write a letter to parents educating them about head lice. The teacher explains that parents are either not detecting the head lice or not telling the school when their child has head lice.

Therapeutic guidelines from a reference book

Head lice (*Pediculus humanus var. capitis*):

Head lice common in school-age children, essentially harmless.
Crawling insects approx size of a sesame seed; live on scalp, lay eggs on hair.
Infestation not indicator of poor hygiene.
Acquired by direct head-to-head contact.
Specific human pathogen; most cases asymptomatic.

Presentation: Eggs (nits) noticed attached to the hair or problem itching of the scalp and nape of the neck.
Possibly presents with excoriations and papules around the occiput (back of the head) and nape, and lymphadenopathy with or without secondary bacterial infection.

Diagnosis of active lice infestation:

Observation of live, moving louse on scalp.

Method:

Apply generous amount of hair conditioner to dry hair; stuns lice for approx 20 mins.
Wet comb hair with fine-toothed comb ('nit combs' available from pharmacies).
Detangle hair, divide into 3-4cm sections, comb each section.
Wipe comb on paper towel, check for lice and nits. Repeat twice for diagnosis.

Treatment:

Approx 40% success rate from wet combing every day for 10 to 14 days until no lice found.

Topical insecticides. Currently recommended products:

- 1 Maldison 0.5% or 1% topically, leave for 8 hours
(Malathion) (not to be used in children <6 months)
OR
- 2 Permethrin 1% topically, leave for a minimum of 20 minutes (not to be used in
children <2 months)
OR
- 3 Pyrethrins 0.165% + piperonyl butoxide 1.65% to 4% topically, leave for a
minimum of 20 minutes

All lice treatments should be repeated after 7 to 10 days, verified next day by conditioner and combing method.

Between treatments use combing method twice; remove all eggs (<1.5cm from scalp) with lice comb or fingernails. May contain viable larvae.

Wet comb weekly for several weeks after cure to detect recurrence.

Precautions and follow-up treatment:

Wash hands thoroughly after lice treatment.

Do not blow-dry hair.

Treat children under 2 years only with medical supervision.

Wash pillow cases on hot cycle; combs and brushes in hot soapy water (60°C).

Examine and treat family and close physical contacts if live lice found.

Notify patient's school, exclusion not necessary after initial treatment.

Nits on the hairs >1.5cm from the scalp = previous infestation, no longer active.

Resistant head lice:

A

- 1 Repeat treatment using another insecticide (see above)

OR

- 2 Wet combing method (see above)

Combing easier with shorter hair styles, but shaving head not necessary

B

If this fails, a doctor may recommend the use of Trimethoprim + sulfamethoxazole (child: 2+10mg/kg up to) 80+400mg orally, 12-hourly for 3 days. Repeat after 10 days.

Note: effectiveness of trimethoprim+sulfamethoxazole thought to be due to the destruction of symbiotic bacteria in the gut of the louse.

Writing task:

Using the information provided, prepare a letter for parents at the local school, Riverside Primary School, educating them about head lice and ways of treating them. Start the letter with 'Dear Parent'.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180 - 200 words.

Occupational English Test

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES

WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:

Today, 1 March 2010, Mrs Daniels, a 78-year-old customer, has come to your community pharmacy asking for something to treat an itchy rash which has been troubling her for the past few days. The rash is quite evident, consisting of large, raised areas which are not red, except where Mrs Daniels has recently scratched them. The rash is mainly on her trunk with some involvement of the upper arms and legs. Mrs Daniels mentions that she has not been gardening, is not prone to rashes, and has no idea about the cause of the rash. When questioned, she says that she has not changed her diet or used different washing powders, soaps, deodorants or perfumes in the last few months.

You note that Mrs Daniels started taking a recently released ACE inhibitor, 'Drug X', about two weeks ago, after suffering side effects with verapamil, which she was taking for hypertension.

Medication history for Mrs Daniels:

09/01/10	Nitrazepam	5mg	1noct
26/01/10	Verapamil	80mg	1b.d.
11/02/10	Indapamide	2.5mg	1q.d.
17/02/10	'Drug X'	5mg	1b.d.

PRODUCT INFORMATION FOR 'DRUG X'

Description:	'Drug X' is a highly specific competitive inhibitor of angiotensin I converting enzyme, the enzyme responsible for converting angiotensin I to angiotensin II.
Pharmacology:	The mechanism of action of 'Drug X' has not yet been fully elucidated; however, it appears to lower blood pressure through suppression of the renin-angiotensin-aldosterone system. This results in a decrease in angiotensin II, subsequent to ACE inhibition, and an increase in plasma renin activity.
Indications:	Hypertension Cardiac failure
Warnings:	Proteinuria has been reported in a small number of patients with pre-existing renal impairment. Neutropenia/Agranulocytosis has occurred in a small number of patients. Hyperkalemia may occur – potassium supplements and potassium diuretics must be used with caution. Cough – a persistent, non-productive cough reported in some patients.
Precautions:	Hypotension may occur occasionally within first three days of therapy. Hypersensitivity reaction – rashes, often associated with pruritis and occasionally fever, may occur. Impaired renal function – increases in BUN and serum creatinine have occurred during treatment.

Adverse Reactions:

More common reactions:

Hypotension

Dermatological — Rash occurred in 10% of patients. Usually pruritic and macro-papular, but rarely urticarial. Generally occurs during the first four weeks of treatment. Usually self-limited and reversible and may respond to antihistamine therapy.

Gastrointestinal — A small number of patients reported loss of taste sensation.

Less common reactions:

Cardiovascular — Tachycardia, chest pain

Dermatological — Angiodema of the face

Gastrointestinal — Nausea, vomiting, pain and constipation may occur

Other — Paraesthesia of the hands

Presentation: 5mg & 10mg oral tablets

You suspect an ADR (adverse drug reaction) to the ACE inhibitor and decide on the following course of action:

Advise Mrs Daniels to visit her doctor as soon as possible.

Notify Mrs Daniels' doctor, Dr J Sotto, of Newtown.

Provide immediate treatment with half percent hydrocortisone cream.

Report the suspected ADR to the relevant authority: the Adverse Drug Reactions Data Bank in Centreville.

Writing task:

Using the information in the notes, write a letter to the Registrar of the Adverse Drug Reactions Data Bank, Centreville, reporting the suspected ADR and all relevant information. Start the letter with 'Dear Sir/Madam'.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180 - 200 words.

Occupational English Test

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES

WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:

You are a pharmacist in a community practice. Recently (3 February 2010) you dispensed a prescription for 'Drug X' for Mrs Ellen White.

Mrs White brought the medication back to the pharmacy shortly after it was dispensed because it was out of date. The manufacturer's expiry date on the cardboard carton was 'December 2009'. You apologised, and immediately replaced it with current stock from your shelves. However, Mrs White said that she would complain to the Pharmacy Board about the incident.

The Pharmacy Board is the authority which registers and administers affairs regarding pharmacists in the state in which you live. Its role is to ensure safety and fairness for the public. Complaints against pharmacists are directed to the Board.

In your logbook you wrote these notes for 3/2/10.

New shop assistant (Sarah) started today.

v. busy day too (change in weather perhaps)

I forgot to check expiry date on tablets dispensed to a customer!

Must set up system asap to make sure this doesn't happen again:

- regular checking of expiry dates on all stock (every month)*
- marking anything with approaching expiry date clearly (red sticker?)*
- reminding staff on duty to check carefully*

OET Victory with Dr Ashgan

You have received the following letter from the Pharmacy Board.

Pharmacist
City Pharmacy
6 Main Road
Newtown

10 February 2010

Dear Pharmacist

The Board has received a complaint from Mrs E White regarding the supply of some out-of-date 'Drug X' tablets.

She claims that, on 3 February 2010, you supplied fifty 'Drug X' tablets to her from a prescription written by Dr Williams of the Newtown Central Medical Clinic. She further claims that the tablets supplied had expired in December 2009.

You are requested to explain to the Board:

- whether you dispensed out-of-date medication to Mrs White.
- the action you took when Mrs White returned to the pharmacy with the tablets.
- the action you have taken to ensure that out-of-date stock is detected and not supplied to the public again.

Your response is required in writing by 10 March 2010, and will be considered by the Board at its next meeting.

Yours sincerely

Ms A Seaborn
Director, Pharmacy Board

Writing task:

Using the information in the notes, write a letter to the Board. Include a response to each item the Board has requested information about. Address your letter to Ms Anne Seaborn, Director, Pharmacy Board, Newtown.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180 - 200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

Ms Stokes presented at the pharmacy today complaining of difficulty swallowing her prescribed medication. She wants her scripts to be changed without going to the doctor.

Patient Details:

Name: Ms Marion Stokes

Address: 22 Walker St, Hillcrest

Marital Status: Widowed

DOB: 24.09.1937 (Age 77)

Allergies: Codeine

Current Medication: Mirtazapine 30mg 1 nocte – for depression
Somac 40mg 1 daily – for stomach acid
Metoprolol 25mg bd – BP
Caltrate 600mg 1 daily – for bones

Reason for Presenting:

- Trouble swallowing medication recently prescribed by Dr.
- ↑ in depression.
- Osteoporosis (20 years).
- ↓ weight (lost 8kg in 3 weeks, weighs 53kg).
- Has impaired hearing.

Suggested Solutions:

Change medication regimen to easy form to swallow:

- Avanza 30mg solu tabs – soluble mirtazapine.
- Somac 40mg can come in sachets.
- Metoprolol can be crushed.
- Calcium – chewable or powder form.

OET Victory with Dr Ashgan

- Other Suggestions Given:**
- Offered to write to Dr: request fax new scripts to the pharmacy with above suggested changes to medication regimen, Pt wants meds filled ASAP.
 - Pt to include liquid nutritional drinks in diet (e.g., Ensure Plus).
 - Pt to see nutritionist.
 - Pt to chew food very well (?see a Speech Pathologist re: improve swallowing technique).
 - Pt to see Dr again to check cause of swallowing difficulty, Wt loss, discuss depression.

Writing Task:

Using the information given in the case notes, write a letter to Ms Stokes' doctor, Dr Smart, 11 Francis St, Hillcrest. Outline the problems Ms Stokes is experiencing and inform the doctor of changes you recommend to her prescriptions and the suggestions you have made.

In your answer:

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the notes below and complete the writing task which follows.

Notes:

Mr Sam Grant is in his early forties, and has been a customer of yours for several years. His wife has brought in a new prescription for you to dispense. As Mr Grant is not present, his wife requests a letter with any information her husband should know.

Prescription:

Dr Philip Goodwin 45 Main Street Sandhurst Park	
Mr Sam Grant 23 Steven's Road Sandhurst Park	Crestor (rosuvastatin calcium) 10mg 30 tabs 1 tablet daily

Prescribing information for drug - Crestor (rosuvastatin calcium)

- Actions:**
- Works by blocking an enzyme in the liver
 - Helps lower LDL (bad cholesterol) produced by the liver
 - Raises HDL (good cholesterol)
 - Diet, weight control and exercise can slow build-up of plaque in the arteries
 - More potent at lowering LDL-C on a milligram-for-milligram basis compared with other statins
- Precautions:**
- Notify doctor before taking Crestor if patient has any of the following:
 - Liver problems
 - Kidney problems
 - Low thyroid hormone levels
 - Unexplained pain in their muscles
 - Regularly drink large amounts of alcohol
 - History of muscle disorders
 - History of muscle problems from using other lipid-lowering drugs
 - Do not stop taking Crestor before first discussing it with doctor. Take Crestor regularly to get best benefit
 - Avoid drinking alcohol: can increase triglyceride levels, may damage liver
 - Care should be taken while driving a car or operating machinery until known if Crestor has an effect on patient
 - May cause dizziness in some people

- How to take it:**
- Administered orally
 - Take Crestor once a day, at the same time each day
 - Swallow each tablet whole with a full glass of water
 - Can be taken with or without food
 - If dose missed, take it as soon as possible
 - Never take 2 doses of Crestor within 12 hours of each other

NOTE: a low-fat diet and regular exercise should be maintained in addition to taking Crestor

In the event of an overdose:

- Telephone doctor or Poisons Information Centre
- Go immediately to Emergency at nearest hospital even if no signs of discomfort or poisoning

Use in pregnancy & lactation:

- If you become pregnant, stop taking Crestor and see doctor immediately
- Do not use Crestor if trying to become pregnant or breast feeding

Side effects:

- Stop taking Crestor and call doctor at once if any of these serious side effects appear:
 - Muscle pain, tenderness, or weakness with fever or flu symptoms and dark coloured urine
 - Urinating more or less than usual, or not at all
 - Nausea, stomach pain, low fever, loss of appetite, clay-coloured stools, jaundice (yellowing of the skin or eyes)
 - Chest pain
 - Swelling in hands or feet

Less serious side effects may include:

- | | |
|-------------------------------------|----------------------------------|
| - Weakness, dizziness | - Pain or burning when urinating |
| - Memory loss | - Constipation, diarrhoea |
| - Sore throat, runny or stuffy nose | - Headache |

Benefit:

- Lowering cholesterol can help prevent heart disease and hardening of the arteries – These can lead to heart attack, stroke and vascular disease

Additional Information:

- Can take several weeks for medication to improve condition
- Liver and blood need to be tested regularly
- Lower dosage may need to be prescribed

Writing Task:

Using the Information given in the case notes, write a letter to Mr Sam Grant of 23 Steven's Road, Sandhurst Park. Advise him on how to take his new medication, precautions, its benefits and any side effects he needs to be aware of.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME:** **5 MINUTES**
 WRITING TIME: **40 MINUTES**

Read the case notes and complete the writing task which follows.

Notes:

You are a pharmacist at Newtown Pharmacy.

Client Details:

Name	Mr Pablo Martinez
DOB	13.01.43
Address	Newtown Retirement Village, 100 Rose Avenue, Newtown
Social History	Retired bookkeeper Wife died 4 months ago Lives in retirement village (Independent housing, staff supervision minimal)
Allergies	Nil

Medical Conditions:

Hypertension:

- Diagnosed: about 10 yrs ago.
- Diet/lifestyle management: poor.

Type 2 diabetes:

- Diagnosed: 8 years ago.
- Diet/lifestyle management: poor.

Anxiety:

- Newly diagnosed; difficulty coping after wife's death (4 months ago).
- Formerly dependent on wife for activities of daily living (ADLs) and help with remembering medications.
- Feels anxious living on his own.

Medications and Treatment Record:

Diabetes	Daonil (glibenclamide), a sulfonylurea – 10mg orally with food once daily. • Commenced: 2 years ago. • Compliance: generally good. Recently (since wife's death), client reports poor compliance – forgetting to take medication at same time each day; skipping meals. • Coincides with episodes of hypoglycaemia: client relies on 'emergency' jelly beans or honey.
Hypertension	Hygroton (chlorthalidone) a thiazide diuretic – 25mg orally daily, in morning with food. • Long term prescription. • Compliance: generally good. Recently (since wife's death) client reports skipping tablets.
Anxiety	Client presents today (23/05/2015) with new script from family doctor (Dr Ellen Casey – Newtown Medical Centre): Inderal (propranolol), a beta-blocker – 40mg bd.
Problem:	Potential drug interaction – Inderal (beta-blocker) and Daonil (oral hypoglycaemic): Caution during hypoglycaemic episode – beta-blocker masks effects of low blood sugar (e.g., disguises sweating and tachycardia).

Poor medication compliance:

- Requires help for ADLs, difficulty eating, remembering medications.
- ?closer supervision in residential aged care facility needed (for more dependent clients, more staff supervision).

Plan:

- Counsel on lifestyle / exercise / diet.
- Refer to GP (family doctor – Dr Casey).
- Caution on Inderal – suggest substitution with antidepressant (e.g., SSRI).
- Recommend Home Medication Review (to address poor medication compliance).

Writing Task:

Using the information provided in the case notes, write a letter to Mr Martinez's family doctor, Dr Ellen Casey, summarising the client's history and outlining your concerns about his medication management. Address the letter to Dr Ellen Casey, Newtown Medical Centre, 90 Central Avenue, Newtown.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME:** **5 MINUTES**
WRITING TIME: **40 MINUTES**

Read the notes below and complete the writing task which follows.

Notes:

You are the pharmacist at Best Care Pharmacy, Greywalls. There has been an outbreak of scabies at a local primary school (for children aged 5-12).

INFORMATION (from one of your reference books)

What it is:	An infestation caused by the parasitic mite, <i>Sarcoptes scabiei</i> , which measures 0.2-0.4mm in length.
Who it affects:	People of all ages, races and socioeconomic groups worldwide; not related to standards of personal hygiene.
People most at risk:	People with weakened immune systems, elderly.
How it is spread:	Direct, personal skin-to-skin contact with infected person (not animals). Easily spread to household members, people in childcare centres/residential facilities. Also spread by sharing clothing, towels, and bedding.
What happens:	The female mite burrows just below top layer of skin to lay eggs. She then dies. The eggs hatch into tiny mites after 10-14 days. These spread out over the skin and live about 30days-2months. The excreta of the mites cause an allergic rash.
Main symptoms:	• Greyish tunnels and/or erythematous papular rash, especially in natural creases of skin such as the webbing between the fingers; wrists; elbows. • Intense itching (worse with warmth and at night) over most of the body. • Sores on the body caused by scratching; sores can sometimes become infected by bacteria.
When symptoms appear:	Around 4-6 weeks after contact in people who haven't had scabies before.
Infectious period:	Until 24 hours after treatment is commenced. The scabies mite can live for 2-3 days on clothes, bed linen and other personal items of infected people.
Treatment:	Scabicide lotions or creams (available from pharmacist), e.g., permethrin preparations (e.g., Lyclear cream or Quellada lotion) or benzyl benzoate 25% preparations (e.g., Ascabiol, Benzemul 25% – diluted for children aged 2-12 and adults with sensitive skin with water). For children under 2 months of age, sulfur 5% cream or crotamiton 10% cream (e.g., Eurax). No prescription required.

Treatment Notes:

- All household members should be treated simultaneously even if no symptoms.
- Use according to the manufacturer's directions.
- Itching may persist for 2-3 weeks after the treatment, but improvement should occur within a few days.
- Skin lesions can take up to 2 months to disappear completely.
- Re-visit doctor or pharmacist 4 weeks after the treatment.
- Antihistamines, calamine lotion, moisturiser for itchiness.
- Antibiotics for secondary bacterial infection from scratching.

Preventing the spread of scabies

- Maintain good personal hygiene.
- Avoid sharing clothes, towels or bed linen with others.
- Remove affected children from school and childcare centres until end of infectious period.
- Avoid close contact with infected individuals.
- The scabies mite can live on bed linen, clothes, towels:
 - Wash patient's bed linen, etc. in hot water and hot tumble dry.
 - Blankets: dry-clean or place in a tumble dryer on a hot setting for ½ hour.
 - Alternatively, kill scabies mites by sealing items in a plastic bag for 1 week before laundering.
- No need to treat furniture or rooms with insecticide.

Writing Task:

Using the information given in the notes, write a letter to the parents of the children at West Street Primary School, Greywalls, advising them of the scabies outbreak, the risk of contagion, symptoms, appropriate measures to avoid the spread of the condition, and what to do in case of infection.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

You are a pharmacist at Newtown Hospital. Mrs Paloma is undergoing chemotherapy and radiotherapy for breast cancer.

PATIENT DETAILS:

Name: Mrs May Paloma

DOB: 29.11.1988

Address: 81 Wind Street, Newtown

Social history: Married – husband (Mr Luke Paloma)

Medication history:

12/12/14	Zofran 4mg (ondansetron) for nausea
13/01/15	Zofran 4mg repeat
12/02/15	Imodium 2mg (loperamide) for gastroenteritis
16/03/15	Zofran 4mg repeat
	Serepax 15mg (oxazepam) for anxiety/difficulty sleeping
19/04/15	Serepax 15mg repeat
	Unisom Sleepgels 50mg (diphenhydramine) for difficulty sleeping

Prescribing doctors:

General Practitioner (GP)

Dr Paul de Luca, GP

Newtown Medical Clinic, 92 Green Street, Newtown

Oncologist Dr June Windslow

Newtown Hospital, 613 Main Street, Newtown

Allergies: Eggs

Treatment Record

12/12/14 Prescription Zofran 4mg prescribed by Dr Windslow (Oncologist).
Directions: Take ONE three times daily for 1-2 days after chemotherapy.
Pt advised to contact prescribing Dr If:

- Continues to vomit after taking the antiemetic.
- Vomits 4-5 times in 24 hour period.
- Has pain in stomach before nausea & vomiting occurs.

13/01/15 Zofran repeat.

OET Victory with Dr Ashgan

11/02/15	Client called Pharmacy Department still experiencing vomiting 24hrs after chemotherapy treatment – wants recommendations for another antiemetic. Advice given: vomiting may be caused by other reasons – client to contact GP.
12/02/15	Prescription Imodium 2mg by GP for gastroenteritis – the cause of vomiting.
15/02/15	Client returned to Pharmacy Department. Asked about side effects of Imodium and chemotherapy. Wants to cease Imodium. Symptoms: dry mouth, gas. Advice given: contact GP before ceasing medication.
16/03/15	Zofran 4mg repeat. New medication added by Oncologist: Serepax 15mg for anxiety and difficulty sleeping. Directions: Take 1 tablet up to TWICE a day and 1 at night only if needed.
19/04/15	Serepax 15mg (from Oncologist) repeated. New prescription (from GP): Unisom Sleepgels 50mg – client still not sleeping despite taking Serepax, went to GP for medication – did not tell the GP about Serepax. Discussions with client: • Potential for interaction between Serepax/Unisom. • Suggested client see Oncologist again. • Client requested I write to Oncologist. Script for Unisom not dispensed.
Plan:	Letter to Oncologist re: Serepax not working effectively and risk of interaction with Unisom.

Writing Task:

Using the information provided in the case notes, write a letter to Mrs Paloma's Oncologist, Dr Windslow, suggesting a review of the client's current medication regime. Address the letter to Dr June Windslow, Newtown Hospital, 613 Main Street, Newtown.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

Mr Henry Styles is to be discharged from the hospital where you work as a pharmacist. He is a 62-year-old male who has been in hospital overnight. While in hospital, he was diagnosed with angina and has been prescribed new medication. Mr Styles has asked you for some written information about angina and his new medication.

His discharge script is shown here:

Discharge Prescription	25 July 2015
Mr Henry Styles	
36 Station Street, Oldmeadows	
Anginine tablets (glyceryl trinitrate): ½ to 1 tab s/l prn x 100	
	Dr R Rich
	District Hospital

Reference books provide the following information about angina and its treatment:

Angina

Background:

- Results from severe coronary obstruction
- During exertion, e.g., exercise, blood supply cannot meet the ↑demand → angina
- Angina ↓ once exertion ceases
- Symptoms include pain located in the chest area
- Risk factors: smoking, hypertension, obesity, diabetes, hyperlipidaemia

Treatment:

(Acute attack)

Glyceryl trinitrate 300-600mcg s/l prn (as needed)

Drug Information reference texts provide the following information about the medication:

Glyceryl trinitrate	
Dose forms available:	Anginine/Lycinate: sublingual (s/l) tablets Nitrolingual: sublingual spray
Strength:	600mcg/tablet 400mcg/dose spray
Indications:	Treatment of acute angina attack
Action:	Vasodilator providing exogenous source of nitric oxide
Pharmacokinetics:	Glyceryl trinitrate has a short half-life in the body Sublingual administration improves rate of absorption and reduces first pass metabolism
Pregnancy:	Safety not established: category B2
Adverse reactions:	Most adverse reactions due to vasodilator properties Common: headaches, flushing, palpitations, orthostatic hypotension, syncope, peripheral oedema Infrequent: rebound angina
Dosage and administration instructions (for tablets):	Administer sublingually when chest pain occurs, allow to dissolve Sit or lie down before use – risk of hypotension • Use ½ tablet sublingually • If pain is not relieved in 5 minutes, administer second ½ tablet sublingually • In five minutes if pain still present, call an ambulance and administer a whole tablet sublingually
Storage Conditions (for tablets):	• store in original bottle (do not transfer to other containers) • do not store close to body as heat → evaporation of drug • keep away from moisture, heat and light • discard tablets 90 days after opening bottle due to loss of potency

Writing Task

Using information given above, write a letter to Mr Styles, providing information on angina and use/storage of his medication. Address the letter to Mr Henry Styles, 36 Station Street, Oldmeadows.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the notes below and complete the writing task which follows.

Notes:

You are a pharmacist in Stillwater Suburban Pharmacy. You have been approached by the manager of Stillwater Soccer Club, Mr Gibbs, for information about nutritional supplements containing bicarbonate and ribose.

Sports Supplements – General Information

- Safety not guaranteed
- Not all ingredients always listed on labels
- Claims of improved performance often not supported by scientific evidence
- Some contain banned substances – would result in disqualification for athletes in competition
- Effective supplements produce particular effects and suit particular sports
- Some enhance aspects of athlete performance with appropriate use
- Proper supervision strongly recommended

Notes from Bicarbonate (HCO_3^-) Information Sheet

General Information

- Provides primary fuel source for exercise of near maximal intensity (lasting longer than approx 20–30 secs)
- Limited by progressive $\uparrow$ in acidity of intracellular environment
- Should delay onset of muscular fatigue during prolonged anaerobic metabolism
- Used by athletes competing in high-intensity competition events lasting 1–7 minutes (e.g., 400–1500m running, 100–400m swimming, kayaking, rowing and canoeing events)
- May have benefits for prolonging intermittent high-intensity activities (e.g., team sports)
- Has a moderate effect in enhancing performance of anaerobic exercise/events
- Effects may vary in individuals as well as specific sports events
- Only to be used under supervision of sports supplement representative/coach
- Should be practised in appropriate training sessions to assess benefits & side effects

Concerns Associated with Supplement Use

- Reports of gastrointestinal distress – may be reduced if supplementation stopped one day before event
- Interaction with other supplements (e.g., caffeine, creatine)

Interactions:

- Antacids, aspirin, benzodiazepines, flecainide, iron, ketoconazole, lithium, methenamine, methotrexate, quinine, sulfa-containing antibiotics, tetracycline, vitamins

Contraindications:

- High blood pressure, congestive heart failure, kidney disease, pregnancy, planned pregnancy, breastfeeding

Possible side-effects:

- ↑ thirst; stomach cramps; gas

Serious Symptoms (= stop taking supplement):

- Serious headache
- Blood in urine
- Vomit resembling coffee grounds
- Appetite loss
- Irritability
- Weakness
- Frequent urge to urinate
- Slow breathing
- Swelling of feet or lower leg
- Bloody, black or tarry stools

Notes from Ribose (C₅H₁₀O₅) Information Sheet**General Information**

- Ribose is a pentose (5-carbon sugar) naturally found in the diet
- Chemical production of ribose now found in a number of commercial supplements (typically providing doses of 3–5g of this sugar, often in combination with creatine)

Overview

- Said to speed muscle recovery after exercise & limit post-exercise fatigue
- Marketing claims: 'dramatic reductions in recovery from 72 hours to 12 hours'
- Claims not supported by scientific evidence

Concerns Associated with Supplement Use

- Absence of full reports / ribose supplementation studies → don't know reported side effects
- Ribose supplementation expensive (daily dose of 10–20g = \$50–100/week)

Writing Task:

Using the information given in the notes, write a letter to the manager of the Stillwater Soccer Club, Mr Gibbs, about the use and effectiveness of supplements containing bicarbonate and ribose. Address the letter to Mr James Gibbs, Manager, Stillwater Soccer Club, 73 Greenfield Road, Stillwater.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: **READING TIME:** **5 MINUTES**
 WRITING TIME: **40 MINUTES**

Read the case notes below and complete the writing task which follows.

Notes:

You are a pharmacist at Newtown Pharmacy.

Patient Details

Name: Ms Amelia Smith
DOB: 27.07.1983
Address: 97 George Avenue, Newtown
Social History: Single; lives alone
 Currently unemployed – former Marketing Manager (made redundant 8 weeks ago)
Allergies: Gluten intolerance
 Nil known medication allergies

Medication History and Treatment Record:

21/04/15 First visit to this pharmacy
 Oroxine (thyroxine) 100mcg/day
 – First time use for newly diagnosed hypothyroidism
 – Patient (Pt) advised on drug administration and side effects
26/06/15 Repeat script for Oroxine (thyroxine) 100mcg/day
 – Pt reports fatigue though seems to be improving
 – Regular check-ups with GP
31/08/15 Repeat script for Oroxine (thyroxine) 100mcg/day
 – Pt made redundant at work 1 week ago, feels very unhappy.
 Otherwise responding well to treatment.
11/10/15 Pt commenced Prozac (fluoxetine) 10mg once daily
 – Pt still depressed
 – GP recommended medication; Pt worried re: mood altering drugs
 – Pharmacist discussed benefits of medication with Pt including side effects (SEs)

15/10/15	New script for antihistamine Claritin (loratidine) 10mg tab once daily – Consulted doctor 14/10/15 re: skin rash & hives (upper arms and thighs). GP suspected gluten in meal at restaurant on 13/10/15.
17/10/15	Pt seeking stronger antihistamine for worsening symptoms – Rash/hives spread to lower arms and abdomen, painful itching – Dizzy, mild swelling of lips – Very anxious
Problem:	Rash unresponsive to antihistamine, worsening
Diagnosis:	Suspect drug-induced skin rash – ?Prozac SE ?Risk of anaphylaxis → urgent medical attention
Plan:	Stop Prozac immediately Pt told to go straight to hospital Emergency Department Called Newtown Hospital – Doctor-in-Charge requested information in writing

Writing Task:

Using the information provided in the case notes, write a letter to the Doctor-in-Charge, Emergency Department, Newtown Hospital, Newtown. Summarise the patient's history and outline your concerns.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

Jonathon Apple is a ten-year-old boy who has type 1 diabetes (insulin dependent diabetes, IDDM). Jonathon is going away for a week to school camp, where the children will be participating in many activities. You are requested to prepare a letter for his school, outlining information on his condition and his medications. You have the following information available on your dispensing computer and in your reference texts:

Medication List

Brand name	Strength	Drug name	Directions
Lantus vial	100 units/mL	insulin glargine	Inj 10 units b.i.d. nocte
Novorapid Flexpen	100 units/mL	insulin aspart	Inj 5 units b.c. i.d.e. immediately before meals
Glucagon	1mg	Glucagon	1mg b.c. m.d.o.

Type 1 diabetes:

- also known as insulin dependent diabetes
- autoimmune disorder that destroys the beta cells of the pancreas → decreased insulin
- insulin replacement required to preserve life
- treatment for type 1 diabetes is lifelong
- blood glucose monitoring required (up to four times a day) to detect hypoglycaemia or hyperglycaemia

Hypoglycaemia:

- causes:
 - excessive dose of insulin
 - delayed/insufficient food
 - ↑ physical activity
- warning signs: sweating, hunger, tremor, palpitations, tremor, headache, visual disturbance and altered mood
- treatment: glucose (jelly beans, sugary drinks, sugar paste) if conscious
- glucagon administered for severe hypoglycaemia, and seek medical assistance

Hyperglycaemia:

- precipitated by a lack of insulin at times of acute stress or illness
- symptoms: nausea, vomiting, rapid breathing, sweet breath odour
- seek medical attention as soon as possible

Medication Information:

Lantus®

- Description:** Long-acting insulin analogue to replace endogenous insulin production
- Administration:** Injected subcutaneously into abdomen region. Rotate injection sites in same general area to avoid swelling of fatty tissue (lipodystrophy).
- Storage:** Vials should be stored in refrigerator (2-8°C, do not freeze) until opened. Once opened, vial can be kept at room temperature but must be discarded within one month.

Novorapid Flexpen®

- Description:** Rapid acting insulin analogue
- Administration:** Injected subcutaneously into abdomen region. Rotate injection sites in same general area to avoid swelling of fatty tissue (lipodystrophy). Give immediately before meal or, if necessary, after start of meal.
- Storage:** Flexpen should be stored in refrigerator (2-8°C, do not freeze) until pen is first used. Once pen has been used, it may be stored out of refrigerator but then this pen should be discarded within one month.

GlucaGen®

- Indication:** Hypoglycaemia
- Dosage:** Adult, child > 5 years: 1mg SC
- Administration:** Injected subcutaneously into thigh, buttock or upper arm. Vial should be reconstituted with diluent provided, and powder allowed to dissolve before administration.
- Storage:** Store at 25°C. Once reconstituted use immediately.

Writing Task:

Using the information given in the notes, write a letter for Jonathon's school nurse that includes a general description of his condition and the medications he is taking, including any special instructions. Address the letter to: Ms Linda Hatter, School Nurse, Greywalls Primary School, 65 Park St, Greywalls.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.